

Stream — Competitor Battle Cards

For the GTM team — not for distribution to prospects Pricing verified 19 Aug 2026 · re-check quarterly

Every product in this deck is a scribe. That is the whole argument.

We don't win a scribe-versus-scribe comparison. Several are cheaper, three are free, most are more mature, and several have real EHR write-back that we don't have. A rep who fights on transcription accuracy is fighting on the one axis where we're at best even, against companies with more funding and more users.

We win by changing the question from **"which scribe is better"** to **"what happens after the note."** Every card ends in the same place: the competitor's answer is "nothing, it's in your EHR."

THE THREE QUESTIONS THAT WORK ON ANY OF THEM

1 "What happens to the note after you paste it?"

Every scribe's honest answer is "nothing." Text in a field. Nothing organized, nothing carried forward, nothing queryable.

2 "Who reads the faxes?"

No scribe in this deck touches inbound documents. This is the pain nobody is selling to.

3 "How do you know which of your patients are overdue for something?"

A spreadsheet, a report nobody opens, or nobody. Again — no scribe addresses it.

Ask these instead of pitching. They're diagnostic, not rhetorical — if the prospect has good answers, we may genuinely not be a fit, and you want to know that in minute five rather than minute forty.

WHERE STREAM SITS, FOR REFERENCE

STREAM PRO \$149 / clinician / mo	FREE TIER No — 30-day trial	EHR WRITE-BACK None
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Three things we never claim on a competitive call.

Never "integrates with your EHR," "syncs," or "writes back." We removed these from the website in August 2026 because they weren't true. Freed Premier, Suki and Tali *do* have EHR push — if we claim it too, we lose the credibility that makes our version land. Say: *works alongside any EHR, nothing to install, no IT project.*

Never SOC 2. We don't have it. Say what's true: BAA on every plan, per-tenant database isolation, role-based access, audit logging, two-factor. Then move.

Never invent a time-saved statistic. Use the named references and 30,000+ documented encounters.

Freed

The default recommendation in independent-primary-care circles. Our most-encountered head-to-head.

PRICE \$39 · \$79 · \$119	FREE TIER No	EHR WRITE-BACK Premier only
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Wins with: solo and 1–3 provider practices who want a scribe and nothing else.

Their pitch: fast, cheap, dead simple, signed up in five minutes.

WHERE THEY BEAT US

- ▲ \$79 unlimited against our \$149. A real gap — don't pretend otherwise.
- ▲ Almost nothing to learn. Our seven-step setup is heavier.
- ▲ Maturity and word of mouth.
- ▲ Premier pushes to the EHR. We don't.

WHERE WE WIN

- Their output is a document. Ours is a chart — problem cards that persist, Timeline and Recap per problem.
- No answer at all for inbound documents, tasks, care gaps or HCC.
- Two years of Freed is ~10,000 unstructured notes and no structure.

THE QUESTION

“After two years and ten thousand Freed notes — what can you actually do with them?”

If they already use it: don't attack it. It's a good scribe and saying otherwise costs you the room. Position Stream as what Freed never tried to be — *“Keep the note fast. The question is what the chart looks like in year three.”* Follow up with objection-map category 1, ★ *The Note Is Solved. The Chart Is Not.* Full comparison page at </comparison/freedai/> — send after the call, not during.

Heidi

The hardest card in the deck. “Why pay \$149 when Heidi is free?” is a fair question and needs a real answer.

PRICE \$0 · ~\$150	FREE TIER Yes — real	EHR WRITE-BACK Not on Free or Clinician
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Wins with: price-sensitive clinicians, and anyone wanting zero-risk trial.

Their pitch: genuinely free to start, strong templating, big international footprint.

WHERE THEY BEAT US

- ▲ A real free tier — unlimited basic consults and dictation.
- ▲ Template flexibility and a polished product.

WHERE WE WIN

- Free Heidi is a scribe with no chart. Same category argument, at \$0.
- Their Clinician plan is now ~\$150 — **the same as Stream Pro**. At parity the comparison stops being about money.
- No inbound documents, no longitudinal problem chart, no care-gap surfaces.

THE QUESTION

“If the free one does what you need, what's still taking up your evenings?”

On price, concede it openly: *“If all you want is a note, take the free one — genuinely. We're not a cheaper scribe, we're a different product. The question is whether the chart is worth solving too.”* Conceding is what makes the rest credible. **Watch:** their Feb 2026 rise from ~\$99 to \$150 annoyed a lot of small practices — anyone who budgeted at \$99 is now a switcher.

Twofold

Targets our exact ICP — “independent practices and outpatient clinics” — at a third of our price, with coding bundled.

PRICE \$49 · \$69	FREE TIER No	EHR WRITE-BACK Claims yes
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Wins with: independent practices and outpatient clinics. Also sells into therapy.

Their pitch: unlimited everything, transparent pricing, CPT/ICD-10 included, BAA, style learning.

WHERE THEY BEAT US

- ▲ Price, badly. \$49 against \$149 — and coding is bundled.
- ▲ No audio stored — a clean privacy answer for cautious buyers.
- ▲ They claim EHR integration. We have none.

WHERE WE WIN

- A very good, very cheap note generator — and nothing else. No problem chart, no document pipeline, no care-gap or HCC surfaces.
- They're competing on price in a market where price is collapsing. A race we shouldn't enter and they can't exit.

THE QUESTION

“At \$49 unlimited, what happens to the notes after they're written?”

Handle the gap head-on. Don't defend \$149 as a scribe price — it isn't one and the prospect can do arithmetic. *“If you want the cheapest good scribe, it's probably them. We're priced like a chart because that's what we are.”*

Also a marketing problem: most “best AI scribe 2026” and competitor-pricing pages ranking in our category are Twofold's own content, including pages targeting rivals by name. A prospect who “did some research” has likely read their framing of the landscape.

Doximity Scribe

Free to verified US clinicians — which is essentially every physician you'll call.

PRICE Free	FREE TIER Entirely free	EHR WRITE-BACK None — copy/paste
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Wins with: anyone who already has a Doximity account. Which is everyone.

Their pitch: free, trusted brand, already in your pocket.

WHERE THEY BEAT US

- ▲ Free, with zero acquisition friction — no procurement, no new vendor.
- ▲ Brand trust. Doximity isn't a startup to them.

WHERE WE WIN

- They copy-paste too — so the EHR objection is **neutral here, not a loss.**
- English only.
- Note generator and nothing more. The recording is discarded after the summary — nothing ever accumulates.

THE QUESTION

“When you use Doximity Scribe, where does the note go?”

Don't fight the price. You can't beat free with a discount. Play category: free solves the note, nothing solves the chart. If they only want a note, Doximity is a fine answer — let them have it. A graceful exit gets you referred; a forced trial gets you a churn.

Abridge & Microsoft/Nuance DAX Copilot

Different companies, same sales situation. Usually “my friend at the hospital uses it,” not a competing quote.

PRICE ~\$200–\$1,200+	FREE TIER No self-serve at all	EHR WRITE-BACK Yes — deep
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Wins with: health systems and large multi-specialty groups.

Their pitch: deepest EHR integration, KLAS scores, enterprise credibility.

WHERE THEY BEAT US

- ▲ Real bidirectional EHR integration, especially DAX with Epic. Not close.
- ▲ Enterprise credibility, security attestations, procurement-ready.
- ▲ Very large deployments with published outcomes.

WHERE WE WIN

- An independent 4-clinician practice generally **cannot buy either one** — no self-serve, no published price, implementation built for a health system.

THE QUESTION

“Did they quote you, or is that what the hospital across town uses?”

It's almost always the second, and the conversation ends there. If they genuinely have a quote, they're bigger than our ICP and likely have an IT department — that's a disqualify. Say so honestly and don't burn the hour.

Suki

The only card in the deck where Stream is the lower-priced option — roughly half.

PRICE ~\$299-\$399+	FREE TIER No	EHR WRITE-BACK Yes
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Wins with: small and mid practices who want EHR connectivity and will pay for it.

Their pitch: ambient scribe plus voice commands, with EHR integrations.

WHERE THEY BEAT US

- ▲ EHR integrations we don't have.
- ▲ Voice command surface.

WHERE WE WIN

- **Price — lead with it.** Roughly half.
- Still a scribe. Same category argument, at 2–3× our price.

THE QUESTION

“At \$300 a month, what are you getting beyond the note?”

Tali AI

Free, natively integrated, *and* government-funded. Three structural disadvantages at once, and we have none of the three.

PRICE \$0 · CAD \$100	FREE TIER Yes	EHR WRITE-BACK Canadian EMRs, live
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Wins with: Canadian primary care. Effectively the default there.

Their pitch: Canadian-built, integrated into the EMRs Canadian clinics actually run.

WHERE THEY BEAT US

- ▲ Live integration with Oscar Pro, Accuro, PS Suite, Med Access, CHR, Healthquest, Profile.
- ▲ **Canada Health Infoway is funding up to 10,000 one-year licences** for eligible primary care clinicians.
- ▲ Free tier on top of that, plus tasks, forms, clinical search, call-a-patient.

WHERE WE WIN

- Still a scribe plus utilities — not a longitudinal problem chart with document ingestion.
- Our HCC and risk-adjustment surfaces are US-specific and don't translate. The win has to be made on chart structure and inbound documents alone.

THE QUESTION

“What happens to the note after it lands in Oscar?”

The honest read: don't prospect into Canada right now. Take Canadian inbound and sell the chart and the fax pile — but free, natively integrated and publicly funded is not a market to spend outbound effort on.

The EHR's own scribe

Elation Note Assist · eCW Sunoh · Athena ambient. The one competitor that wins the double-documentation objection outright — and the one nobody puts on a competitor list, because it doesn't feel like a company.

PRICE Bundled / near-free	FREE TIER Already paid for	EHR WRITE-BACK Native
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Their pitch: it's already in your EHR. Nothing to buy, nothing to log into.

WHERE THEY BEAT US

- ▲ Zero extra login, vendor or invoice.
- ▲ The note lands in the chart natively. No paste. **We lose this objection outright.**
- ▲ Procurement is a checkbox.

WHERE WE WIN

- Note quality is usually noticeably worse — bolt-ons, not the company's core product.
- It writes into the EHR's flat, document-shaped chart. Nothing organized by problem, nothing carried forward, faxes still a pile.
- No care-gap surface a clinician sees *at the visit*.

THE QUESTION

“Pull up your EHR's recall report while we're on the call — does it know the difference between a visit that's clinically due and one your payer will actually cover yet? And does it reach the clinician at the visit, or land in a report nobody opens?”

Be honest about the paste. Two sentences — one click, formats into any field, section by section — then move on. Arguing a fair objection makes it bigger.

Status quo

Still the most common competitor, and the easiest room. No feature comparison — only inertia and AI skepticism.

WHAT YOU'RE UP AGAINST

- ▲ "We've always done it this way."
- ▲ Burned by an earlier AI tool, or by hype generally.
- ▲ "Too busy to switch right now."

WHAT TO DO

- **Skeptic** → objection-map category 5. Lead with provenance: every claim from an outside document links to the exact sentence it came from, and anything unverifiable is suppressed. **Show it, don't describe it.**
- **Burned out** → category 2.
- **Too busy** → 30-day trial, no card, nothing to install.

THE QUESTION

"What does documentation cost you after clinic ends?"

When to walk away — and what's missing

DISQUALIFY FAST

Reps who disqualify quickly close more. Hand it back if:

- **They require EHR write-back as a condition.** We don't have it and won't this year. Say so plainly.
- **Health-system owned, or they have an IT department.** Not our product.
- **They need SOC 2 to sign.** We don't have it. Don't promise a date.
- **Under 3 clinicians.** Route to the self-serve trial per the booking floor — not a loss, a different door.
- **They only want a note and are happy at \$0–79.** Heidi Free, Doximity or Freed serve them better today. A clean “we're not your answer yet” earns referrals.

WHAT WOULD MAKE THIS DECK REAL

Everything here is reasoned from our content, public pricing and market structure — **not from our own loss data, because we don't capture any.** As of 19 Aug 2026 there are 94 closed-lost deals in HubSpot and **closed_lost_reason is empty on every one.**

- **Required close-lost reason** with a competitor picklist. Five-minute HubSpot change.
- **Ask “what are you using today?”** on every qualifying call, and log it.

Within a quarter this deck rewrites itself from evidence. Until then the ordering is an explicit hypothesis: Freed, Heidi, Twofold and the EHR-native scribes are probably 80% of what reps actually hit; Abridge and DAX are probably name-drops.

Pricing verified 19 August 2026 from public pricing pages and third-party reviews. Note that Twofold, Commure, Vero and DeepCura all sell scribes themselves — their pricing tables are useful and broadly consistent, their verdicts are not neutral. Verify any number you plan to say out loud against the vendor's own page. Source list in **gtm/competitor-battle-cards.md**.